

Triage Report - T-2317-180885

RED

PATIENT INFORMATION

Age: 55
Sex: Male
Pregnant: No
Chief Complaint: Chest Discomfort

CLINICAL SUMMARY

The patient, a 55-year-old male, presents with severe chest discomfort, pain severity of 8/10, and radiation to the left arm. He reports shortness of breath, sweating, nausea, and no history of heart disease or palpitations. The pain is described as crushing in nature. There are no similar episodes reported.

DIAGNOSIS

Clinical Assessment: Chief Complaint: The patient reports crushing chest pain with radiation to the left arm, shortness of breath, sweating, and nausea. Pain Type: Crushing Pain Location: Center Pain Severity: 8/10 Pain Radiation: Left arm Shortness of Breath: Yes Sweating: Yes Nausea: Yes History of Heart Disease: No Palpitations: No Similar Episodes: No Risk Factors: - Cardiac Event: Possible cardiac event due to severe chest pain with shortness of breath. - Myocardial Infarction (MI): Possible MI due to chest pain radiating to arm. - Cardiac Event: Possible cardiac event due to chest pain with sweating. Clinical Significance: The patient's chief complaint of severe chest pain with radiation to the left arm, shortness of breath, sweating, and nausea is concerning for a cardiac event. The pain severity of 8/10 and the presence of sweating are significant indicators of a potential myocardial infarction (MI). The patient's history of no history of heart disease or palpitations further supports the assessment of a cardiac event. Immediate Actions Needed: 1. Emergency Department Admission: The patient should be admitted to the emergency department for further evaluation and management. 2. Cardiac Monitoring: Continuous ECG monitoring should be initiated to monitor for signs of cardiac arrhythmias or ischemia. 3. Intravenous Access: Intravenous access should be established to administer medications and fluids as needed. 4. Lipid Profile: A lipid profile should be ordered to assess for any abnormal lipid levels. 5. Blood Gas Analysis: A blood gas analysis should be performed to assess for any signs of hypoxia or acidosis. Follow-Up Requirements: 1. Follow-Up: The patient should be followed up with a cardiologist within 24-48 hours for further evaluation and management. 2. Re-evaluation: The patient should be re-evaluated in 2-4 weeks to assess for any changes in symptoms or cardiac status. Triage Priority: Red

KEY CLINICAL FLAGS

Chest pain with shortness of breath - possible cardiac event (RED)
Chest pain radiating to arm or jaw - possible MI (RED)
Chest pain with sweating - possible cardiac event (RED)

TRIGGERED RISK RULES

chest_pain_with_sob: Chest pain with shortness of breath - possible cardiac event
chest_pain_radiation: Chest pain radiating to arm or jaw - possible MI
chest_pain_sweating: Chest pain with sweating - possible cardiac event
chest_pain_severe: Severe chest pain (8-10 on pain scale)
default_standard: No high-risk factors identified - standard assessment needed

CONCLUSION & RECOMMENDATIONS

The patient's severe chest pain with radiation to the left arm, shortness of breath, sweating, and nausea is a critical presentation that requires immediate medical attention. The patient should be admitted to the emergency department for further evaluation and management. Cardiac monitoring, intravenous access, and lipid profile should be initiated as soon as possible. Follow-up with a cardiologist is recommended within 24-48 hours and re-evaluation in 2-4 weeks is necessary.

PATIENT INSTRUCTIONS:

Please proceed immediately to the emergency area. A staff member will assist you.